

REFERRAL PACKET

When chronic symptoms don't respond to standard care, the jaw is often the missing variable.

A clinical and scientific orientation for referring physicians, dentists, and allied practitioners.

Dr. Dwight Jennings runs a tertiary referral practice in the San Francisco Bay Area focused on jaw orthopedics and the cranio-facial / systemic-symptom interface. Patients arrive with chronic headaches, fibromyalgia, sleep apnea, TMJ dysfunction, undiagnosed pain syndromes, mood and movement disorders that have not improved under mainstream care. Many fly in from across the country.

The framework his practice is built on rests on a single neuropeptide — **substance P** — and a chain that connects jaw position to systemic inflammation via the trigeminal nerve. The framework is unusual in dental medicine. It is not unusual in the substance P literature.

“In 2024, Elsevier published a 28-chapter reference textbook on substance P. Every condition I treat that mainstream medicine struggles with has a dedicated chapter in that book. I'm not making a novel claim. I'm connecting an established neuropeptide to an established list of conditions through a specific mechanical cause.” — Dr. Jennings

This packet contains: (1) the mechanism in one page, (2) the conditions where a referral is most likely to help, (3) the 2024 textbook chapters that back the framework for each condition, and (4) how to refer a patient.

THE MECHANISM IN ONE PAGE

How a misaligned jaw becomes a systemic problem.

Four steps. The first three are established neuroscience. The fourth maps to Dr. Jennings' clinical observation across forty years.

1. Structural · Jaw misalignment

A vertical-height deficiency — often inherited, often worsened by extraction-based orthodontics — forces the lower jaw to retrude to bring the teeth together. The TMJ is compressed. The chewing muscles (especially the temporalis) work continuously to hold the jaw in a non-resting position.

2. Neural · Trigeminal hyperactivation

Chronic muscle and joint irritation activates the trigeminal nerve — the body's densest concentration of nociceptive C-fibers. C-fibers release **substance P**, a neuropeptide with no reuptake mechanism. It accumulates and lingers.

3. Biochemical · Substance P at the cell

Substance P binds the NK-1 receptor, opens cell membranes, lowers cell voltage, and allows calcium influx. NF-κB — the master inflammatory regulator — is activated. Substance P is established as a primary driver of inflammatory disease in respiratory, gastrointestinal, and musculoskeletal systems (O'Connor et al., *J Cell Physiol* 2004; Vink 2024 Ch 13).

4. Systemic · Downstream dysfunction

Sustained substance P elevation correlates clinically with the conditions Dr. Jennings sees most often: chronic headaches, fibromyalgia, IBS, autoimmune disorders, sleep apnea, movement disorders, mood and ADD/ADHD presentations, and a broader pattern of complex chronic illness. Whether substance P is causal or only correlative in each is an active research question. The clinical observation — that bite alignment therapy reduces this constellation in patients who haven't responded to mainstream care — is what the practice was built on.

The treatment is undramatic — almost never surgery. A precision-fitted removable dental appliance is used to reposition the lower jaw to a physiologically optimal rest position. Substance P production downregulates. In responders, the downstream symptoms reduce or resolve.

CONDITIONS, MAPPED TO THE 2024 LITERATURE

When to consider a referral.

Each block below pairs a presenting pattern with the chapters in Substance P (Vink R, ed. Elsevier 2024; ISBN 978-0-443-22194-1) that cover the underlying mechanism in the formal literature.

TMJ PAIN & DYSFUNCTION

TMJ pain & dysfunction

Jaw pain, clicking, locking, ear pressure, facial pain, post-surgical TMJ complications. The original specialty of the practice.

Ch 15	Substance P in pain SP as a critical player in neuronal sensitization and cortical pain processing — including the emotional aspects of pain.
Ch 24	SP–NK-1R in musculoskeletal disorders Direct treatment of SP's role in myalgia, tendonitis, fascial hypersensitivity. Discusses NK-1R antagonists as candidate therapeutics.
Ch 11	SP in bone metabolism SP regulates osteoclast/osteoblast activity. Aging and disc degeneration correlate with increased SP-expressing osteocytes.

CHRONIC HEADACHES & MIGRAINES

Chronic headaches & migraines

Daily or near-daily headaches, classical migraine, tension-type, cluster — particularly those unresponsive to standard pharmacological care.

Ch 15	Substance P in pain See above — central sensitization mechanism.
Ch 13	Substance P in inflammation SP activates NF-κB via NK-1R; trigeminovascular inflammation is the substrate of most chronic-headache syndromes.

SLEEP APNEA (AND CPAP-INTOLERANT PATIENTS)**Sleep apnea (and CPAP-intolerant patients)**

Particularly indicated when CPAP has failed or been declined, when snoring + retrognathia + chronic fatigue co-occur, or when sleep apnea presents alongside the broader chronic-illness pattern.

Ch 25***SP and sudden infant death syndrome (SIDS)***

SP/NK-1 signaling dysfunction compromises cardiorespiratory reflexes to hypoxia. The infant-mortality literature points at the same pathway.

Ch 10***SP in the central control of respiration***

Direct role of SP in respiratory rhythm and reflex.

MOVEMENT DISORDERS**Movement disorders**

Parkinson's, Tourette's, dystonia, torticollis, gait disorders. Not all cases respond — but a subset have a strong cranio-mandibular component and respond to alignment therapy where other approaches have not.

Ch 20***SP in Alzheimer's and Parkinson's disease***

SP reduced in AD/PD brain regions. SP colocalizes with α -synuclein in PD olfactory bulb, GI tract, vagus nerve. SP proposed as prognostic marker and disease-modifying target.

SURGICAL ALTERNATIVE (TMJ SURGERY / ORTHOGNATHIC SURGERY CANDIDATES)**Surgical alternative (TMJ surgery / orthognathic surgery candidates)**

Patients recommended for joint replacement, condylectomy, or jaw-advancement surgery deserve a non-surgical second opinion before proceeding. Many can be treated non-surgically with precision jaw orthopedics.

Ch 24***SP–NK-1R in musculoskeletal disorders***

Pharmacological and mechanical paths to the same downregulation surgery attempts to achieve.

COMPLEX & UNDIAGNOSED CHRONIC ILLNESS**Complex & undiagnosed chronic illness**

Fibromyalgia, IBS, autoimmune patterns, chronic fatigue, ADD/ADHD, anxiety and depression that don't fit cleanly into a single specialty's workup. Often the patient has seen many physicians and run a long differential.

Ch 24***Musculoskeletal disorders***

Fibromyalgia, fascial pain.

Ch 26***Immunobiology and pharmacology of SP***

SP secreted by immune cells AND responded to by immune cells — the hinge between nervous and immune systems.

Ch 13***SP in inflammation***

IBS, autoimmune mechanism.

Ch 21***NK-1R antagonists for major depressive disorder***

Five clinical trials demonstrating antidepressant efficacy; chapter argues this drug class deserves re-advancement.

Ch 22***Mouse model of ADHD via NK1 receptors***

ADHD phenotype validated in NK1R-knockout mice.

Ch 23***SP in stress and anxiety***

Central SP elevation is anxiogenic; NK-1R blockade is anxiolytic.

Ch 27***SP in viral diseases — esp. COVID-19***

Relevant for post-COVID symptom syndromes.

HOW TO REFER

Sending a patient to Dr. Jennings.

There is no formal referral form. A short note from you, plus the patient calling to schedule, is enough.

What to send

If you have any of the following, they help us assess fit before the first visit: panoramic dental imaging, recent MRI or CT of head/neck, sleep study, current medication list, and a brief narrative of what has been tried.

Who is a good fit

Patients who have plateaued under conventional care and where the presentation includes any one of: TMJ pain or dysfunction; chronic headache or migraine; bite that has shifted with age or after orthodontics; chronic-illness pattern with a strong substance-P-mediated footprint (fibromyalgia, autoimmune, mood, ADD, post-viral); sleep apnea with retrognathia or CPAP intolerance; or any movement disorder where a trigeminal-mandibular contribution has not yet been ruled out.

Who is not a good fit

Patients seeking cosmetic dentistry, routine restorative work, orthodontics for adolescents without complicating symptoms, or acute dental emergencies. Dr. Jennings does not provide general dentistry.

Logistics

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This packet is informational. Nothing here constitutes diagnosis or treatment advice. Any clinical decision remains with the patient and their care team.